SNAPSHOT

The building, two ways. They are mutually exclusive.

The campus plan lets Alameda 113 sqm and keeps the rest. The medical plan gives Medbury Alameda the whole building. Both cannot happen.

THE CAMPUS MODEL

Alameda is a tenant on 113 sqm. Medbury keeps the rest.

What occupies it	Six businesses. Sessional rooms upstairs for 45 visiting consultants, four clinical OpCos, diagnostics in the chalet, pharmacy in the boys' quarters
Revenue in the building	**NGN 1,598 M**
Campus company EBITDA	**NGN 186 M**
Clinics, Medbury share	**NGN 214 M**
TO MEDBURY, YEAR 5	**NGN 400 M**
Medbury capital	**NGN 523 M**
Return on that capital	**0.8 times a year**
Control	100% of the campus company, 51 to 100% of the four clinical companies

THE MEDICAL PLAN

Medbury Alameda occupies all 664.3 sqm. Medbury owns 49%.

What occupies it	One centre. Ground floor public and diagnostic, first floor clinics, procedures and 9 observation beds, chalet a full laboratory, boys' quarters engineering
Revenue in the building	**NGN 2,954 M**
Campus company EBITDA	**NGN 316 M, rent only**
Centre, Medbury 49% share	**NGN 395 M**
TO MEDBURY, YEAR 5	**NGN 712 M**
Medbury capital	**NGN 578 M**
Return on that capital	**1.2 times a year**
Control	100% of the landlord, 49% of the operating business. A minority of the thing that earns

The medical plan returns more and controls less.

It is NGN 712 M a year to Medbury against NGN 400 M, on NGN 578 M of capital against NGN 523 M. But it is a minority stake in the business that earns it, and it deletes the sessional business, the four clinical companies, and diagnostics and pharmacy as companies Medbury owns outright. Decide which building you are financing before the fit-out is tendered, because the two need different rooms.